

العمر(تاريخ الميلاد) : 29-09-1950 المهنة : ربه منزل

التعليق على التشخيص : -----

رئيس
المجلس الطبي المتخصص

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Name: جيهان مصطفى زوام مختار National ID: 25009290101401	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



جيهان
مصطفى زوام
٣٢ ش المحروسة
العجوزة - الجيزة

١٩٥٠/٠١/٢٩ ٢٥٠٠٩٢٩٠١٠١٤٠١

JP8280279



٢٥٠٠٩٢٩٠١٠١٤٠١
٢٠٢٢/١٠
حاصله على ليسانس حقوق



أنثى
مطلقة
متزوجة
محمود عباس بيومي صبوله
البطاقة سارية حتى ٢٠٢٩/١٠/٢٨





جاءه الاصدار بفرع الجيزة

تاريخ الاصدار: 2023-10-12

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية
وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي
والبحث تبين أن

السيد (ة) جيهان مصطفى زوام بطاقة رقم 25009290101401
وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المنتفعة بالتأمين الصحي صدر هذا الخطاب دون اي مسئولية على الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة: 2023-10-12



إفادة مركز نظم المعلومات
يعتمد،،،



الخطاب ساري لمدة ثلاثة أشهر من تاريخ الإصدار إلا في حالة تغيير حالة المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعي أو وزارة التربية والتعليم

الإدارة المركزية لتكنولوجيا المعلومات و دعم القرار

970

Pat. Name: GEHAN MOSTAFA ZWAM MOKHTAR	Pat. Code: 25009290101401
Study Date 02/05/2026	Rad Dept ID: 1279108

CT SCAN OF THE ABDOMEN AND PELVIS

HISTORY: A known case with a right breast cancer. Compared to the previous CT study, dated 13/12/2025

TECHNIQUE:

Axial contiguous cuts were taken through the abdomen and pelvis with oral and IV contrast injection.

FINDINGS:

- The liver is **still enlarged** in size displaying diffuse decreased attenuation pattern, denoting diffuse fatty infiltration with no evidence of definite focal lesions. No extra or intrahepatic biliary dilatation.
- The gall bladder is surgically removed.
- The PV: is of average diameter with normal contrast opacification with no CT evidence of internal thrombosis.
- The spleen is of average size displaying normal and homogenous CT appearance.
- The pancreas shows normal C.T. appearance.
- Both kidneys are seen in their normal anatomical site displaying average size and normal shape. No evidence of abnormal masses or backpressure changes. Both show adequate contrast excretion being functioning.
- Normal size and CT density of the adrenal glands.
- No sizable abdominal or pelvic adenopathy.
- No evidence of ascites.
- Normal appearance of the urinary bladder with normal wall thickness and no calculi, masses or diverticula.
- Normal CT appearance of the uterus and both adnexal regions.
- **Unchanged non-complicated colonic diverticulosis** is noted. Otherwise, normal CT appearance of the rest of the scanned bowel loops. No pelvic masses or fluid collection.
- Bone window reveals dorso-lumbar spondylodegenerative changes evident by presence of marginal osteophytes yet with no evidence of definite lytic/sclerotic bony lesions by CT criteria. Lower lumbar fixation is noted.
- Aorto-iliac atherosclerotic changes noted evident by presence of multiple scattered calcific plaques involving the abdominal aorta and both iliac arteries.

IMPRESSION: A follow-up study showing a stationary course regarding the followings:

- Fatty hepatomegaly.
- Non-complicated colonic diverticulosis.

Much obliged,

Dr. Ahmed M. Osman, MD

Professor of Radiodiagnosis, Ain Shams University

Electronically Signed By: Ahmed Osman

04/05/2026 19:51

Pat. Name: GEHAN MOSTAFA ZWAM MOKHTAR	Pat. Code: 25009290101401
Study Date 02/05/2026	Rad Dept ID: 1279108

CT. SCAN OF THE CHEST

HISTORY: A known case with a right breast cancer. Compared to the previous CT study, dated 13/12/2025

TECHNIQUE:

Axial contiguous thin cuts were taken through the chest after IV contrast injection.

FINDINGS:

- A **stable** right breast lateral irregular soft tissue mass lesion = 14 x 10 mm associated with right breast skin thickening.
- A **stationary course** is noted regarding the following:
 - ✓ Right upper and right middle lobes peripheral sub-pleural reticular infiltration and fibrosis is noted.
 - ✓ Right lower lobe posterior pulmonary atelectatic bands associated with a few pulmonary cysts.
 - ✓ A left upper lobe posterior solid pulmonary nodule is noted = 4.5 mm.
- Otherwise, both lungs show normal appearance of its parenchyma with no evidence of abnormal masses, other nodules or consolidative patches.
- Normal CT appearance of the mediastinum vessels.
- No sizable mediastinal or hilar adenopathy.
- Patent tracheobronchial tree with no abnormal bronchial dilatation.
- No pleural or pericardial effusion.
- Normal cardiac borders and chambers.
- The lower neck cuts are unremarkable.

IMPRESSION:

- A follow-up study showing a **stationary course** regarding the following:
 - ✓ Right breast lateral irregular soft tissue mass lesion with right breast skin thickening.
 - ✓ Right upper and right middle lobes peripheral sub-pleural reticular infiltration and fibrosis, features of post-irradiation sequel/fibrosis.
 - ✓ Right lower lobe posterior pulmonary atelectatic bands associated with a few pulmonary cysts, likely a sequel of old resolving pulmonary pneumonic process.
 - ✓ A left upper lobe posterior solid pulmonary nodule (4.5 mm), likely non-specific yet regarding the patient's history of cancer breast, follow-up is recommended.
- No newly developed lesions.

Much obliged,

Dr. Ahmed M. Osman, MD

Professor of Radiodiagnosis, Ain Shams University

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name : جيهان مصطفى زوام مختار

Sex:

3- Medical number:

Female

83164610

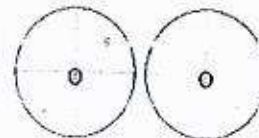
Old Med :

Old Med :

3608-2023

2-ID number:

Patient Age : 74 Y



Family history

Family History

Phone

Phone:

01007329461

BMI

BMI

28

REASON OF MDT

REASON OF MDT
REASON

marital status

Marital Status

NRS Score

NRS Score

3

GROUP

GROUP

C

Date of MDT

Date of MDT

14/10/2024

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

PET CT: residual masses & chest wall elision

BX IDC	ER 7/8	PR 3/8	Her 2 Neu 0	SISH	Ki67 15
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T	N	M 1 soft tissue
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Sonomammography

PATIENT HISTORY

2018 RT CBS +PORTH +tamoxifen

7/2023 chest wall mets biopsy breast cancer luminal A

8/2023 started femara IBRANCE till 10-2024 with excellent response

New PET-CT: SD

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

Surgically, the recurrence is fixed to posterior chest wall and serratus.

Omit surgery in view of possible complications and keep on Faslodex-Ibrance

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ نوري كاسم	RADIOLOGY	RADIOLOGY د/ مروه ابو السعود
SURGERY	SURGERY د/ محمود علي	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition اميرة حجابي

Pat. Name: JIHAN MOSTAFA ZOAM	Pat. Code:
Study Date 21/08/2025	Rad Dept ID: QC2_481439

Bowel

- **Non-complicated diverticulosis**, predominantly involving the **sigmoid colon** — unchanged from prior imaging.
- **No signs of diverticulitis** or obstructive pathology.
- **Normal appearance of the remainder of the bowel loops.**

Bone Structures

- **Dorso-lumbar spine shows degenerative changes**, including marginal osteophytes.
- **No definite lytic or sclerotic bone lesions** noted on CT — **no evidence of osseous metastasis.**

Overall Impression

Stable imaging findings in a known case of treated right breast carcinoma, with:

1. **Stable right anterior-lateral chest wall nodular lesion** (largest 14 × 10 mm).
2. **Unchanged right breast skin thickening** — likely post-treatment scarring.
3. **Stable radiation-induced pulmonary fibrosis** in the right upper and middle lobes.
4. **Stable bulky left adrenal gland** — likely benign, unchanged from prior.
5. **No hepatic, pulmonary, nodal, or skeletal evidence of metastasis.**
6. **Non-complicated sigmoid diverticulosis**, unchanged.
7. **No new or suspicious lesions detected** in the chest, abdomen, or pelvis.

Thank you for your referral

Yours truly,

Dr. Mohamed Teiba, MD

Consultant of Radiodiagnosis, Ain Shams University

Electronically Signed By: Mohamed Teiba
24/08/2025 16:16

Pat. Name: GEHAN MOSTAFA ZWAN	Pat. Code:
Study Date 13/12/2025	Rad Dept ID: QC_585913

CT SCAN OF THE ABDOMEN AND PELVIS

HISTORY: A known case with a right breast cancer. Compared to the previous CT study, dated 21/08/2025

TECHNIQUE:

Axial contiguous cuts were taken through the abdomen and pelvis with oral and IV contrast injection.

FINDINGS:

- The liver is **still enlarged** in size displaying diffuse decreased attenuation pattern, denoting diffuse fatty infiltration with no evidence of definite focal lesions.
- No extra or intrahepatic biliary dilatation.
- The gall bladder is surgically removed.
- The PV: is of average diameter with normal contrast opacification with no CT evidence of internal thrombosis.
- The spleen is of average size displaying normal and homogenous CT appearance.
- The pancreas shows normal C.T. appearance.
- Both kidneys are seen in their normal anatomical site displaying average size and normal shape. No evidence of abnormal masses or backpressure changes. Both show adequate contrast excretion being functioning.
- Normal size and CT density of the adrenal glands.
- No sizable abdominal or pelvic adenopathy.
- No evidence of ascites.
- Normal appearance of the urinary bladder with normal wall thickness and no calculi, masses or diverticula.
- Normal CT appearance of the uterus and both adnexal regions.
- **Unchanged non-complicated colonic diverticulosis** is noted. Otherwise, normal CT appearance of the rest of the scanned bowel loops.
- No pelvic masses or fluid collection.
- Bone window reveals dorso-lumbar spondylodegenerative changes evident by presence of marginal osteophytes yet with no evidence of definite lytic/sclerotic bony lesions by CT criteria. Lower lumbar fixation is noted.
- Aorto-iliac atherosclerotic changes noted evident by presence of multiple scattered calcific plaques involving the abdominal aorta and both iliac arteries.

IMPRESSION: A follow-up study showing a stationary course regarding the followings:

- Fatty hepatomegaly.
- Non-complicated colonic diverticulosis.

Much obliged,

Dr. Ahmed M. Osman, MD

Professor of Radiodiagnosis, Ain Shams University

Pat. Name: GEHAN MOSTAFA ZWAN	Pat. Code:
Study Date 13/12/2025	Rad Dept ID: QC_585913

CT SCAN OF THE ABDOMEN AND PELVIS

HISTORY: A known case with a right breast cancer. Compared to the previous CT study, dated 21/08/2025

TECHNIQUE:

Axial contiguous cuts were taken through the abdomen and pelvis with oral and IV contrast injection.

FINDINGS:

- The liver is **still enlarged** in size displaying diffuse decreased attenuation pattern, denoting diffuse fatty infiltration with no evidence of definite focal lesions.
- No extra or intrahepatic biliary dilatation.
- The gall bladder is surgically removed.
- The PV: is of average diameter with normal contrast opacification with no CT evidence of internal thrombosis.
- The spleen is of average size displaying normal and homogenous CT appearance.
- The pancreas shows normal C.T. appearance.
- Both kidneys are seen in their normal anatomical site displaying average size and normal shape. No evidence of abnormal masses or backpressure changes. Both show adequate contrast excretion being functioning.
- Normal size and CT density of the adrenal glands.
- No sizable abdominal or pelvic adenopathy.
- No evidence of ascites.
- Normal appearance of the urinary bladder with normal wall thickness and no calculi, masses or diverticula.
- Normal CT appearance of the uterus and both adnexal regions.
- **Unchanged non-complicated colonic diverticulosis** is noted. Otherwise, normal CT appearance of the rest of the scanned bowel loops.
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IMPRESSION: A follow-up study showing a stationary course regarding the followings:

- Fatty hepatomegaly.
- Non-complicated colonic diverticulosis.

Much obliged,

Dr. Ahmed M. Osman, MD

Professor of Radiodiagnosis, Ain Shams University

Patient Name : الأستاذة / جيهان مصطفى زوام	Request No. : 13008601
Patient No. : 25009290101401	Request Date : 20/09/2023
Path No. : SR-1109/23	Verification Date : 09/10/2023
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 72 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Slide Revision

Gross Examination:

- One paraffin block labelled by (866/23) was received.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis):(3+2+1) Grade II.
- * DCIS: No.
- * Angioinvasion: No.
- * Perineural invasion: No.
- * IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 70% of tumor cells (Allred score 7/8).
- PR: Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 15% nuclear reaction of tumor cells.

Diagnosis:

Right breast cancer, Slide revision:

Invasive duct carcinoma , grade II.

IPT:

Verified by :

Rana fathy

Patient Name	: الأستاذة / جيهان مصطفى زوام	Request No.	: 13008601
Patient No.	: 25009290101401	Request Date	: 20/09/2023
Path No.	: SR-1109/23	Verification Date	: 09/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 72 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

- ER: Positive (score 7/8).
- PR: Positive (score 3/8).
- Her2: Negative (score 0).
- Ki67: Positive in 15% of tumor cells.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :
Rana fathy

Printing Date : 16/10/2023 13:19:23

Page 2 of 2

Printing By : Dr.Marwa mahmoud

Pat. Name: GEHAN MOSTAFA ZWAM MOKHTAR	Pat. Code: 25009290101401
Study Date 22/09/2026	Rad Dept ID: 1279108

CT SCAN OF THE ABDOMEN AND PELVIS WITHOUT CONTRAST

HISTORY: A known case with a right breast cancer. Compared to the previous CT study, dated 5/2026

TECHNIQUE:

Axial contiguous cuts were taken through the abdomen and pelvis without oral and IV contrast injection.

FINDINGS:

- The liver is **still enlarged** in size displaying diffuse decreased attenuation pattern, denoting diffuse fatty infiltration with no evidence of definite focal lesions (as regard this non contrast study) . No extra or intrahepatic biliary dilatation.
- The gall bladder is surgically removed.
- The PV: is of average diameter
- The spleen is of average size displaying normal and homogenous CT appearance.
- The pancreas shows normal C.T. appearance.
- Both kidneys are seen in their normal anatomical site displaying average size and normal shape. No evidence of abnormal masses or backpressure changes.
- Normal size and CT density of the adrenal glands.
- No sizable abdominal or pelvic adenopathy.
- No evidence of ascites.
- Normal appearance of the urinary bladder with normal wall thickness and no calculi, masses or diverticula.
- Normal CT appearance of the uterus and both adnexal regions.
- **Unchanged non-complicated colonic diverticulosis** is noted. Otherwise, normal CT appearance of the rest of the scanned bowel loops. No pelvic masses or fluid collection.
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- Aorto-iliac atherosclerotic changes noted evident by presence of multiple scattered calcific plaques involving the abdominal aorta and both iliac arteries.

IMPRESSION: A follow-up study showing a stationary course regarding the followings:

- Fatty hepatomegaly.
- Non-complicated colonic diverticulosis.

Much obliged,

Sara Abdelwahab, MD

Electronically Signed By: sara mohamed

Nuclear Medicine Report

Name: السيدة / جيهان مصطفى زوام	Age: 76 yrs
Date : 1 / 10 / 2026	Code: 01279108

Tc-99m MDP Bone Scan

In comparison to previous scan on 18/8/2025 (~ 1 yr)

- ❖ Present scan reveals nearly the same tracer biodistribution as before with no bone abnormality suspected for destructive osteoblastic deposits.
- ❖ Degenerative changes are observed at mid-dorsal and lower lumbar spines with mild scoliosis.
- ❖ Osteoarthritic reactions are still suggested for shoulders (especially Lt one).
- ❖ Rt upper limb edema is depicted by enhanced soft tissue count.
- ❖ Kidneys are smaller than average size.

Signature

Dr. Magdy Shaaban, MD

Magdy Shaaban

1 October 2026, 3:43:05 PM